Obsessive-Compulsive Personality Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

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1. What it is

Obsessive-compulsive personality disorder (OCPD) is a long-standing preoccupation with **orderliness, perfectionism, and mental and interpersonal control**, at the expense of flexibility, openness, and efficiency, beginning by early adulthood (American Psychiatric Association [APA], 2022). It belongs to **Cluster C** (anxious or fearful).

How common: OCPD is the **most common** personality disorder in community surveys: 7.88% of U.S. adults in a large national survey (Grant et al., 2004) and 4.32% across Western community studies (Volkert et al., 2018).

OCPD is not OCD. OCD involves unwanted, distressing obsessions and compulsions that the person usually recognizes as excessive. OCPD is a lifelong style of perfectionism and control that the person often sees as right and reasonable. The two can co-occur, and OCPD also often co-occurs with depression, anxiety, and eating disorders (APA, 2022; Diedrich & Voderholzer, 2015).

Counselor framing: these clients are often conscientious, hardworking, and moral, and they may come for depression, burnout, or marriage conflict. Beneath the control is fear of mistakes, criticism, or chaos.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A pervasive pattern, beginning by early adulthood, with **4 or more** of:

- Preoccupied with **details, rules, lists, order, or schedules** to the point that the main purpose of the activity is lost.
- **Perfectionism that interferes with finishing tasks.**
- Excessively **devoted to work and productivity** at the expense of leisure and friendships (not explained by financial need).
- **Overconscientious, scrupulous, and inflexible** about morality, ethics, or values (not explained by cultural or religious identification).
- **Unable to discard** worn-out or worthless objects even without sentimental value.
- **Reluctant to delegate** or work with others unless they do things exactly his or her way.
- **Miserly** spending style; money is hoarded for future catastrophes.
- Shows **rigidity and stubbornness**.

Note the religious exception: DSM-5-TR specifically says moral and ethical seriousness that reflects the person's culture or religious identity does not count toward the diagnosis (APA, 2022). Devout faith is not OCPD.

When counselors must refer (general clinical consensus): refer for evaluation when there is significant depression, suicidal thinking, OCD symptoms (intrusive obsessions or rituals), an eating disorder, or hoarding that creates safety problems.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in obsessive-compulsive personality disorder. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
SCID-5-PD (Structured Clinical Interview for DSM-5 Personality Disorders)	Diagnosing all ten DSM-5 personality disorders, with an optional self-report screener.	Semistructured clinician interview	Yes	★ Clinical interviewing skill; training recommended	APA Publishing

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
PID-5 (Personality Inventory for DSM-5)	Dimensional personality trait profile (negative affect, detachment, antagonism, disinhibition, psychoticism); brief and informant forms.	Self-report; 220-item full or 25-item brief form	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
FMPS (Frost Multidimensional Perfectionism Scale)	Perfectionism (concern over mistakes, doubts, high standards, organization).	Self-report, 35 items	No (free)	None beyond general clinical training	Original article
Y-BOCS (Yale-Brown Obsessive Compulsive Scale)	Checking whether OCD (unwanted obsessions and rituals) is also present.	Clinician interview, 10 severity items	No (free)	★ Clinical interviewing skill; training recommended	Original article
PAI (Personality Assessment Inventory)	Broad clinical and personality assessment for adults, including scales for borderline and antisocial features.	Self-report, 344 items, about 50 to 60 minutes	Yes	★ Level C	PAR
MCMI-IV (Millon Clinical Multiaxial Inventory-IV)	Adults in treatment; personality patterns alongside clinical syndromes.	Self-report, 195 true/false items, about 25 to 30 minutes	Yes	★ Level C	Pearson
MMPI-3 (Minnesota Multiphasic Personality Inventory-3)	Broad personality and psychopathology assessment, with validity scales for over- and under-reporting.	Self-report, 335 true/false items, about 25 to 50 minutes	Yes	★ Level C	Pearson

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: OCPD-specific research is small. Cognitive therapy, short-term dynamic therapy, and schema therapy all helped in trials of Cluster C personality disorders, which include OCPD.

Evidence overview (honest effect sizes):

- **Cluster C trial:** among 50 patients with avoidant, dependent, or obsessive-compulsive personality disorder, 40 sessions of cognitive or short-term dynamic therapy led to improvement maintained over 2 years; 42% (cognitive) and 54% (dynamic) recovered from symptoms, and about 40% in each group recovered in interpersonal problems and personality functioning (Svartberg et al., 2004).
- **Schema therapy:** in a multicenter trial of 323 patients with Cluster C, paranoid, histrionic, or narcissistic personality disorder, schema therapy led to recovery more often than usual care or another psychotherapy (Bamelis et al., 2014).
- **Cognitive therapy and the alliance:** in an open trial of up to 52 sessions of cognitive therapy for avoidant and obsessive-compulsive personality disorders (30 patients), a stronger early alliance and more repaired "ruptures" in the relationship predicted greater improvement, including in depression (Strauss et al., 2006).
- **Medication (referral notes):** no medication is approved for OCPD; medication may treat co-occurring depression, anxiety, or OCD.

Cognitive work with OCPD: how it is used (Strauss et al., 2006; Bamelis et al., 2014):

- **Weigh costs and benefits** of perfectionism and control in work, health, and relationships, using the client's own goals.
- **Identify and challenge core beliefs:** "Mistakes are intolerable," "If I don't do it myself, it won't be done right," "There is one right way," "Relaxing is wasted time," "I must stay in control or things will fall apart."
- **Run behavioral experiments:** deliberately do a task "good enough," delegate one job, leave a small item imperfect, or schedule rest, then compare predictions with results.

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5. Religion, spirituality, and Christian integration: what the evidence shows

Disorder-specific research. We found **no study of religion or spirituality in OCPD** and no faith-integrated treatment trial (literature search, November 1, 2026). Research on religious scrupulosity concerns OCD rather than OCPD; see the OCD review.

Related evidence. Across 49 studies, positive religious coping went with better adjustment to stress, and negative religious coping (feeling punished by God, doubting God's care) with worse (Ano & Vasconcelles, 2005). Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and produced added spiritual benefits (Captari et al., 2018).

Honest summary for teaching:

1. Cognitive, dynamic, and schema therapies for Cluster C disorders are the evidence base. Faith integration supports them; it does not replace them.
2. There is no research on faith and OCPD. Grace speaks directly to perfectionism, but rigid clients may also turn faith into another set of exacting rules.
3. Serious devotion is not a symptom. Look for rigidity that harms relationships and joy, not for strong convictions.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within cognitive, schema-focused care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for perfectionism and control in spiritual form: "God expects me to get everything right," "Rest is laziness," "My way is the biblical way," "If I don't control my family, God will hold me responsible" (Ano & Vasconcelles, 2005).
- **Challenge (clinical).** Test the cost of rigid rules in relationships and health, and run small experiments with "good enough," delegation, and Sabbath rest (Strauss et al., 2006). **Possible cognitive errors** to watch for, with their definitions from the Belief Counseling Cognitive Errors Checklist and the symptoms they tend to feed:
  - **Perfectionism** — standards no one could meet. *Feeds:* preoccupation with details. *Sounds like:* "Anything less than perfect is unacceptable."
  - **Should statements** — rigid musts and oughts. *Feeds:* rigidity and overwork. *Sounds like:* "Rest is laziness; I should always be working."
  - **All-or-nothing thinking** — no middle ground. *Feeds:* inflexibility. *Sounds like:* "If it's not done my way, it's done wrong."
  - **Over-responsibility** — I must control every outcome. *Feeds:* refusing to delegate. *Sounds like:* "If I don't control it, it will fall apart."
  - **Unrealistic expectations** — expecting too much of myself or others. *Feeds:* conflict with others. *Sounds like:* "Everyone should meet my standards."
  - **Catastrophizing** — expecting the worst possible outcome. *Feeds:* anxiety about mistakes. *Sounds like:* "One mistake will ruin everything."
- **Questions to dispute the belief.** Have the client choose questions from the Questioning False Beliefs form that "speak to" the belief. Selected questions that fit the errors above:
  - "Do I really have to be perfect? Is any human being perfect?" *For:* perfectionism.
  - "Do I have my priorities right?" *For:* should statements.
  - "Are there other ways of looking at this situation?" *For:* all-or-nothing thinking.
  - "Do I need to "let go and let God" work in my life?" *For:* over-responsibility.
  - "Is anyone perfect? Haven't we all sinned and fallen short of the glory of God?" *For:* unrealistic expectations.
  - "Am I blowing things up way out of proportion?" *For:* catastrophizing.
- **Replace (pastoral).** Offer Scripture of grace, rest, and what matters most:
  - "Come to Me, all you who are weary and burdened, and I will give you rest. Take My yoke upon you and learn from Me; for I am gentle and humble in heart, and you will find rest for your souls." (Matthew 11:28–29)
  - "'Martha, Martha,' the Lord replied, 'you are worried and upset about many things, but only one thing is necessary. Mary has chosen the better portion, and it will not be taken away from her.'" (Luke 10:41–42)
  - "Then Jesus declared, 'The Sabbath was made for man, not man for the Sabbath.'" (Mark 2:27). Rules serve people, not the reverse.

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    - "In vain you rise early and stay up late, toiling for bread to eat—for He gives sleep to His beloved." (Psalm 127:2)
- **Faith community (clinical).** Value these members' reliability while encouraging shared leadership, flexibility, and rest. Avoid rewarding overwork as spiritual maturity.
- **Ministers:** model grace over performance, help families affected by rigid control, and refer when depression, OCD symptoms, or burnout appear.

7. Quick classroom takeaways

- OCPD is pervasive perfectionism, orderliness, and control (4 of 8 criteria). It is the most common personality disorder (about 4% to 8%), is not the same as OCD, and religious devotion is excluded as a criterion.
- Cognitive, dynamic, and schema therapies help Cluster C disorders; a strong alliance predicts improvement.
- Replace "Mistakes are intolerable" with the gentle yoke of Christ and the rest He gives.
- Measure perfectionism with the free FMPS, and use the Y-BOCS to check whether OCD is also present.

Sources note

All 9 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

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